

Against Medical Advice (AMA) Discharge

Policy Number	POL-PC-065	Owner / Department	Medical Staff / Nursing
Effective Date	2023-06-15	Version	2.1
Last Revised	2026-03-08	Next Review	2028-03-08
Approved By	Medical Executive Committee	Applies To	All inpatient units and the Emergency Department; all licensed providers and nursing staff

1. Purpose

To respect a patient's right to refuse treatment and leave the facility while ensuring the decision is informed and made with capacity, and that the patient's safety and continuity of care are protected, consistent with patient-rights standards (42 CFR 482.13) and accepted against-medical-advice practice.

2. Scope

Applies when a patient with decision-making capacity chooses to leave or to decline recommended care before the provider advises discharge. A patient who leaves without staff knowledge and who is unsafe to be unsupervised is managed as an elopement under POL-SEC-010, not as an AMA discharge.

3. Definitions

- **Against Medical Advice (AMA)** — a patient leaves or refuses recommended care against the provider's advice.
- **Decision-making capacity** — the ability to understand the condition and the risks of leaving, appreciate how they apply, reason about options, and communicate a choice.
- **Informed refusal** — the patient's documented decision to decline care after being informed of the risks, benefits, and alternatives.

4. Procedure

IMPORTANT — Before a patient leaves AMA, a provider must assess decision-making capacity. A patient who lacks capacity cannot be discharged AMA; take steps to ensure safety, involve the surrogate decision-maker, and consult the attending (and Security/Risk Management as needed). Never withhold stabilizing or emergency care because a patient intends to leave.

1. Assess and document decision-making capacity for the specific decision to leave.
2. Explain, in understandable terms, the risks of leaving, the expected benefits of staying, and the alternatives; answer questions without coercion.
3. Offer continued care, follow-up, prescriptions, and a clear invitation to return at any time.

4. Ask the patient to sign the AMA form; if they decline to sign, document that the risks were explained and the patient refused to sign — the discharge is still documented as AMA.
5. Notify the attending provider; file an event report per POL-RM-013.

5. Severity & Reporting

Events are classified and reported under POL-RM-013. A routine AMA departure by a patient with capacity after informed refusal is a SEV-3 event, reported within 24 hours. An AMA departure by a patient with a serious unstabilized condition, or where capacity is in question, is SEV-2 and is reported within 4 hours — notify the attending provider and Risk Management. If the patient lacks capacity and is at risk of harm, escalate as SEV-1 and treat a disappearance as an elopement under POL-SEC-010.

6. Documentation Requirements

- Capacity assessment and its basis; who performed it.
- Specific risks of leaving that were explained and the patient's understanding.
- Alternatives and follow-up offered (medications, return precautions, appointments).
- Whether the AMA form was signed or signing was declined; provider notification.

7. References

- CMS Conditions of Participation, 42 CFR 482.13 — Patients' Rights (right to refuse treatment).
- The Joint Commission, Rights-and-Responsibilities standards on informed refusal.
- Related: POL-SEC-010 (elopement), POL-PC-066 (informed consent), POL-RM-013 (severity & reporting).